

IMEDICAL,

For Surgery Use ☐ Urgent ☐ Ring Patient ☐ Make Appointment ☐ Note in Chart ☐ File ☐

Patient **SCHLEGER, SHANE**
Patient Address **IMEDICAL,1 UNION ST PYRMONT NSW 2009**
Sex **M** Age **40 years** DOB **01/01/1985**
Report For **IMEDICAL,**
Ref. by/copy to **IMEDICAL,**

UR No.

Requested 14/09/2025
Collected 16/09/2025 12:00 AM
Reported 17/09/2025 04:00 PM

292795

CUMULATIVE SERUM THYROID FUNCTION TESTS

Date	16/09/25
Time	00:00
Lab No	29809325
TSH	1.9 mIU/L (0.50-4.00)
free T4	18 pmol/L (10-20)
free T3	5.4 pmol/L (2.8-6.8)

29809325 The FT3 result was obtained by an alternative method.

Clinical Notes : nil

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	Prolactin	258	mIU/L	(< 300)
-	Luteinizing Hormone	< 1	IU/L	(1-10)
	Follicle Stimulating Hormone	1	IU/L	(1-10)
++	Oestradiol	217	pmol/L	(< 150)
+	Progesterone	3	nmol/L	(< 3)
++	Testosterone	47	nmol/L	(10.0-33.0)
+++	free Testosterone (calc)	1183	pmol/L	(150-700)
	Sex Hormone Binding Globulin	32	nmol/L	(13-71)

Clinical Notes : nil

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ADRENAL STUDIES

Serum Cortisol **430 nmol/L**
Collection time: Not Recorded

Cortisol reference intervals reflect the expected diurnal variation:

7:00-9:00 AM: 145-619 nmol/L
3:00-5:00 PM: 95-462 nmol/L

Please note that results from samples collected outside these specified time windows should be interpreted within the clinical context, as cortisol concentrations can vary considerably throughout the day.

Please note that as of 21/07/2025, a reformulated cortisol assay has been implemented. Reference intervals have been updated accordingly

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CUMULATIVE SERUM HIGH SENSITIVITY C-REACTIVE PROTEIN (CRP)

Date	16/09/25
Time	00:00
Lab No	29809325

CRP 0.4 mg/L (0.0-6.0)

C-reactive protein (CRP) is a non-specific indicator of tissue damage. Common causes of markedly increased CRP include infection, trauma, myocardial infarction, malignancy and inflammation.

In apparently healthy men and women who have an intermediate risk of cardiovascular disease, as assessed by major risk factors, CRP can identify a higher risk subgroup with CRP > 3 mg/L.

Range(mg/L)	Risk Estimate
Up to 1.0	Low
1.0 to 3.0	Average
3.1 to 10.0	High
Over 10.0	Assess for acute inflammation

In known, stable, coronary disease a CRP > 1 mg/L has shown increased risk.
Reference: Circulation 2003;107:499-511 & 2007;115:1528-1536

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DHEA - Sulphate 7.2 umol/L (2.2-13.8)
(DeHydroEpiAndrosterone)
(previous units) 2700 ng/mL (800-5100)

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CUMULATIVE LIPID RISK REPORT

Date

16/09/25

Time

00:00

Lab No

29809325

FASTING

Target if
HIGH RISK

Total Cholesterol

4.5 mmol/L (below 4.0)

Triglycerides

0.9 mmol/L (below 2.0)

CHOLESTEROL FRACTIONS

HDL

1.10 mmol/L (above 1.0)

LDL (calculated)*

2.99 mmol/L (below 2.5)

Non-HDL cholesterol*

3.40 mmol/L (below 3.3)

Total/HDL ratio**

4.1

* Secondary prevention LDL and non-HDL cholesterol targets are lower.

** The ratio is for use with the cardiovascular risk calculator.

Web-search: "Australian cardiovascular risk calculator"

29809325 Treatment is recommended if clinically indicated or if calculated risk exceeds 15% absolute risk of CVD events over 5 years.

NVDPA 2012 **Target** ranges refer to **HIGH RISK PATIENTS**.

As of 7/3/22 LDL will no longer be measured routinely. LDL results will be calculated, in accordance with National harmonisation.

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IRON STUDIES

Serum Iron	12	umol/L	(10-33)
Transferrin IBC	46	umol/L	(45-70)
Transferrin Saturation	26	%	(16-50)
Serum Ferritin Assay	68	ug/L	(30-320)

Clinical Notes : nil

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Growth Hormone	< 0.2	mIU/L	(< 19.0)
IGF-1 (Somatomedin C)	24	nmol/L	(14-42)

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SERUM CHEMISTRY - FASTING

Sodium	143	mmol/L	(137-147)
Potassium	4.2	mmol/L	(3.5-5.0)
Chloride	104	mmol/L	(96-109)
Bicarbonate	29	mmol/L	(25-33)
Other Anions	14	mmol/L	(4-17)
Glucose	4.9	mmol/L	fasting (3.0-6.0)
Urea	6.5	mmol/L	(2.5-8.0)
Creatinine	109	umol/L	(60-130)
eGFR	73	mL/min	(over 59)
Uric Acid	0.43	mmol/L	(0.12-0.45)
Total Bilirubin	10	umol/L	(2-20)
Alk. Phos.	74	U/L	(30-115)
Gamma G.T.	10	U/L	(0-70)
ALT	24	U/L	(0-45)
AST	22	U/L	(0-41)
LD	166	U/L	(80-250)
Calcium	2.37	mmol/L	(2.15-2.60)
Adjusted for Albumin	2.28	mmol/L	(2.15-2.60)
Phosphate	1.1	mmol/L	(0.8-1.5)
Total Protein	69	g/L	(60-82)
Albumin	46	g/L	(35-50)
Globulins	23	g/L	(20-40)
Cholesterol	4.5	mmol/L	(3.6-6.7)
Triglycerides	0.9	mmol/L	fasting (0.3-2.2)

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Serum Magnesium 0.9 mmol/L (0.7-1.1)

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FULL BLOOD EXAMINATION

Haemoglobin	169	g/L	(135-180)
Red Cell Count	5.8	x10 ¹² /L	(4.2-6.0)
Haematocrit	0.51		(0.38-0.52)
Mean Cell Volume	88	fL	(80-98)
Mean Cell Haemoglobin	29	pg	(27-35)
Platelet Count	304	x10 ⁹ /L	(150-450)
White Cell Count	7.6	x10 ⁹ /L	(4.0-11.0)
Neutrophils	52 %	4.0 x10 ⁹ /L	(2.0-7.5)
Lymphocytes	37 %	2.8 x10 ⁹ /L	(1.1-4.0)
Monocytes	7 %	0.5 x10 ⁹ /L	(0.2-1.0)
Eosinophils	4 %	0.30 x10 ⁹ /L	(0.04-0.40)
Basophils	0 %	0.00 x10 ⁹ /L	(< 0.21)

Automated Comment:

As per ISLH guidelines - Film not reviewed. If a film review is truly indicated, contact the laboratory within 24 hours of collection. Otherwise investigate any highlighted abnormalities as clinically appropriate.

All haematology parameters are within normal limits for age and sex.

**** FINAL REPORT - Please destroy previous report ****

Clinical Notes : nil

Pathology Report